



Health & Social Services Department

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2200

EFFECTIVE DATE: 20 APRIL 1998

APPROVED BY:

A handwritten signature in dark ink, appearing to read "Thomas L. Charron".

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

A handwritten signature in dark ink, appearing to read "Allen Morini, D.O.".

ALLEN MORINI, D.O., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: EMS QUALITY IMPROVEMENT PLAN

AUTHORITY: HEALTH & SAFETY CODE 1797.204, 1797.220; ADMINISTRATIVE CODE, TITLE 22, DIVISION 9, CHAPTER 4, SECTION 100141.2, 100166

PURPOSE/POLICY:

The Quality Improvement Plan of the Solano EMS System shall complement and be integrated into the Q.I. Plan of each organization and agency that participates in the EMS System. The Enclosed Q.I. Plan shall be the minimum prescribed standards of Q.I. activities that EMS Service Providers shall be responsible for within the Solano EMS System.

I. INTRODUCTION

- A. The local EMS Agency shall establish a system-wide Quality Improvement Program as defined in Section 100142.2 of Title 22, Division 9, Chapter 4 of the California Code of Regulations. Additionally, each EMT-P Service Provider and each EMT-P Base Hospital shall have a Quality Improvement Program approved by the local EMS Agency.

II. SEMSC QUALITY IMPROVEMENT MISSION

The mission of the Solano Emergency Medical Services Cooperative in its role as the Local EMS Agency is to insure the immediate availability of the highest quality appropriate emergency medical care in accordance with National, State and local standards to all people in the communities of Solano County nonprejudicially and regardless of their ability to pay.

III. GOALS

- A. Deliver optimal prehospital patient care.
- B. Assure optimum clinical and field performance of all care-givers within the local EMS System.
- C. Comply with all legislated requirements of California Administrative Code 100141.2, 100166 c.

IV. OBJECTIVES

- A. High quality care.
- B. Consistency in treatment by providers.
- C. Open honest dialogue among EMS providers, base hospitals, and the SEMSC.
- D. Accurate data collection.
- E. Individual and system improvement including tracking and trending.
- F. Provide and promote high quality continuing education.
- G. 100% compliance with licensure, accreditation and certification requirements.
- H. Promote customer focus.
- I. EMS providers and the EMS agency will have active involvement in the committee process.

V. EMS SYSTEM QUALITY IMPROVEMENT ACTIVITIES

The Quality Improvement Plan will insure the following activities are occurring at the level of the EMS Agency, Base Hospitals and EMS Service Providers.

- A. The Continuous Quality Improvement (CQI) Committee will insure that EMS services are consistent with medical expectation and SEMSC policy.
- B. The CQI Committee will define standards of care objectively and establish systems to monitor care and define trends.
- C. EMS Agency will promote active participation at base station education tape review meetings.
- D. EMS Agency will review and revise policies and protocols on an as needed basis.

- E. EMS Agency will promote peer review at the provider level.
- F. EMS Agency will review MIS data gained and periodically report back to the EMS Providers.
- G. EMS Agency will disseminate SEMSC memorandums to all EMS providers in a timely manner.

VI. ORGANIZATION, ACCOUNTABILITY AND RESPONSIBILITY

A. Organization and Accountability.

The Quality Improvement Plan addresses all of the components in the Emergency Medical Services System. This includes the base hospitals, receiving facilities, dispatch agencies and all agencies/companies providing prehospital field care within the local EMS system. Each of these system participants are responsible for coordinating the day-to-day quality assurance activities within their respective areas and integrating quality assurance issues that effect overall emergency medical care in the County.

B. Responsibilities – Local EMS Agency (LEMSA)

1. The Local EMS Agency, as designated by law, is responsible for planning, implementing, monitoring and evaluating the local EMS System.
2. The Medical Director or his/her designee of the EMS Agency is responsible for medical control and medical direction of the local EMS System.
3. The EMS Administrator of the EMS Agency has responsibility for integration and coordination of all system-wide Quality Programs in the EMS System.
4. The EMS Prehospital Care Coordinator is responsible for carrying out the day-to-day activities of the Quality Improvement Program in the Agency. These responsibilities include:
 - a. Acting as primary staff for the Quality Improvement Program.
 - b. Providing technical assistance in Quality Improvement to all system components;
 - c. Identifying and participating in the resolution of important prehospital care issues;
 - d. Collecting and analyzing data;
 - e. Identifying system issues and needs and design and implement mechanisms needed for change;
 - f. Regulating the flow of prehospital patient care issues while insuring reporting through appropriate channels.
5. The Base Hospital(s) and Pre-hospital care provider(s) have the responsibility to implement the policies and procedures of the EMS Agency. This shall include meeting or exceeding minimum quality improvement standards as approved by the EMS Agency, active review of

the services and care provided, and timely submission of productivity reports to the EMS Agency.

6. All EMS personnel providing prehospital care are responsible for meeting or exceeding minimum quality improvement standards as approved by the EMS Agency.

VII. COMPONENTS OF THE QUALITY IMPROVEMENT PROGRAM

- A. The EMS QUIP uses an integration of the EMS providers' QI program and base hospital QI program to gather information and to send items to the QI committee for analysis and recommendation. The EMA Agency will use the following components to improve patient care:
 1. Plan
Utilizes information gathered from QI programs to identify areas needing improvement.
 2. Do
Implement changes that were recommended during the planning stage.
 3. Study
Study a statistically significant sample of the changed process for a specified amount of time.
 4. Act
Implement revised policy/protocol based on the results of the study. See reference.

VIII. METHODOLOGY FOR QUALITY IMPROVEMENT ASSESSMENT

- A. Assessment for Quality Improvement occurs in 3 levels.
 1. Prospective Assessment is preventative planning provided by system planning, policy and protocol development, training and continuing education. Many administrative bodies are involved, including the State Emergency Medical Services Authority (EMSA), the local EMSA (SEMSC), the Health Officer for Solano County, Base Hospital personnel and Pre-Hospital training institutes. Coordination for this function occurs at the local EMSA.
 2. Immediate Assessment occurs by Base Hospital contact during an EMS call. Quality Improvement programs, conducted by the Base Hospital, are directly related to the provision of direct Medical Control.
 3. Retrospective Assessment consists of reviews of various aspects of EMS after the care has been rendered. This may be in the form of Quality Improvement audits, tape review sessions and educational programs. Initial screening takes place at the appropriate EMS Agency and then audit forms are forwarded to the local EMSA for further review, data entry and statistical trend analysis.
- B. The task of coordination of Quality Improvement activity belongs to the Solano EMS Agency. Through contractual agreements and as required by regulations, Base Hospital Liaisons, Provider Q.A. Liaisons, the Continuous Quality

Improvement Committee (CQI), the Pre-Hospital Care Committee (PCC) and Physician's Forum and all EMS providers must participate in the Q.I. process in conjunction with the EMS Office.

- C. Base Hospital Liaison nurses have a specific role in ALS activities review. Audits are done following the criteria set forth by Solano EMS Agency. Issues and identified needs involving medical control and patient outcome will be addressed at this level (see Policy #2201).
- D. Each EMS Prehospital Care Provider shall have a Q.I. Liaison that shall assist both the Base Hospital Liaisons and the EMS Office in the evaluation of unusual occurrences, follow-up training and any other Q.I. issues as delegated by the EMS Agency.
- E. Each EMS Service Provider Organization shall have a Q.I. Plan which is approved by the EMS Medical Director, in writing, and which shall address at a minimum:
 - 1. Productivity – as it relates to that specific EMS System Service and elements of prehospital care where the Service Provider has direct control and the capability of making changes.
 - a. To include data collection and retrospective review (audit) of EMS patient care provided and timely submission of Q.I. Productivity Reports to the EMS Office (as referenced in Solano EMS Agency Policy #2203).
 - b. Records monitoring for certification/accreditation requirements of personnel within their organization.
 - c. In-house education/training programs to meet and exceed LEMSA requirements and other training requirements established by the EMS Medical Director, EMSA or the State Legislature.
 - 2. Prevention/long range planning – as relates to training and projections for future needs.
 - a. To include update and remedial education as required by audit processes.
 - b. To include provisions for education for new skills and procedures.

Reference:

- 1. MANAGING CHANGE TO IMPROVE EMERGENCY MEDICAL SERVICES.
A User's Guide for the California EMS Quality Improvement Model.

SOLANO EMERGENCY MEDICAL SERVICES COOPERATIVE

Board of Directors

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Solano County

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Jendy A. Getty
Assistant County Counsel

Date: 12/12/03

To: ALS Provider CQI Coordinators
Base Hospital Coordinators

From: Michael Frenn, EMS Agency Administrator 

Re: Policy Memorandum For Policies 2201 and 2203

In an effort to clarify process regarding Quality Assurance, the following shall be immediately in effect:

1. Issues may identified by either a base hospital coordinator or an ALS service provider coordinator.
2. When an issue is initially reported, either to an ALS service provider coordinator from a base hospital coordinator, or to a base hospital coordinator from an ALS service provider coordinator, the receiving party shall have 30 days to investigate and report back to the originator. This report may be either verbally or in writing, but should be to the mutual satisfaction of both parties.
3. Issues that are in excess of 90 days from occurrence shall be forwarded directly to the EMS Agency for review and direction.
4. It shall be the responsibility of the party receiving the initial report of an issue to obtain the records necessary to review the issue.

This policy memorandum should be attached to Policies 2201 and 2203 until such time as those policies are rewritten to incorporate these changes. A clean copy of this memorandum will be sent to you under separate cover.

Please feel free to contact me with any questions or concerns.

Cc: CQI Committee



Health & Social Services Department

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2201

EFFECTIVE DATE: 20 APRIL 1998

APPROVED BY:

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THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

A handwritten signature in dark ink, appearing to read "Allen Morini, D.O.", written over a horizontal line.

ALLEN MORINI, D.O., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: EMS QUALITY IMPROVEMENT (Q.I.) PLAN – BASE HOSPITALS

AUTHORITY: HEALTH & SAFETY CODE 1797.204, 1797.220; ADMINISTRATIVE CODE, TITLE 22, DIVISION 9, CHAPTER 4, SECTION 100141.2, 100166

PURPOSE/POLICY:

To establish specific Quality Improvement Requirements for the Base Hospital components of the Solano EMS System.

I. INTRODUCTION

- A. Each Base Hospital servicing Solano County shall have a quality improvement program approved by the Local EMS Agency.
- B. Each Base Hospital servicing Solano County shall have a written agreement to participate in the County's ALS program and to comply with all applicable State Regulations and local Policies and Procedures including participation in the Local EMS Agency's Q.I. System.
- C. Each Base Hospital providing on-line medical control within the Solano County EMS System shall maintain a comprehensive Quality Improvement Program designed to interface with the Local EMS Agency's Quality Improvement Program.

1. GOALS

- a. Delivery of optimal pre-hospital care.
- b. Assure optimum clinical performance of caregivers within the base hospital EMS System.

2. OBJECTIVES

- a. High quality care.
- b. Consistency in Medical Control by individuals within the Base Hospital.
- c. Open, honest dialogue among EMS Providers, Base Hospitals and SEMSC.
- d. Accurate data collection.
- e. Individual and system improvement, including tracking and trending.
- f. Provide and promote high quality continuing education.
- g. 100% compliance with licensure, accreditation and certification requirements.
- h. Promote customer focus.
- i. Base hospitals will have active involvement in the committee process.

II. BASE HOSPITAL Q.I ACTIVITIES

Quality Improvement duties shall be accomplished and shared by the Base Hospital Liaison and Medical Director for each Base Hospital. These individuals shall perform, at a minimum, the following functions:

- A. Audits of pre-hospital calls to include, but not limited to the following:
 - 1. Patient contact by Air Ambulance.
 - 2. All calls that required ALS intervention prior to Base contact or where Base contact was not made.
 - 3. Incomplete documentation on the MICN form.
 - 4. Difference between pre-hospital assessment versus M.D. diagnosis/impression.
 - 5. Additional items as mutually agreed upon by the Base Hospital and the EMS Agency.
- B. These audits shall include visual review of records and as appropriate, audio review of radio tapes as specified in Solano County EMS Policy #7100.
 - 1. Additional Quality Improvement Activities will include:
 - a. Investigation of all unusual occurrences as referred by the EMS Agency. Investigations shall take no longer than five (5) working days or a limited time mutually agreed upon by the Base Hospital and EMS Agency and the results shall be reported directly back to the EMS Agency.

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- b. Weekly review of pre-hospital medical records to insure PCRs are completed correctly and medical records are timely received and appropriately distributed.
 - c. Perform all required orientations of new EMT-P's and MICN's as specified in Solano County Policy #3600.
 - d. Assure attendance of a Base Hospital representative at Pre-Hospital Critical Care Committee (PCC), Continuous Quality Improvement (CQI), and Emergency Medical Care Committee (EMCC) meetings as required by the EMS Agency.
 - e. Infection Control – Base Hospitals servicing Solano County shall develop and strictly enforce policies for infection control and contaminated materials disposal to decrease the chance of communicable disease exposure as well as ensure appropriate follow-up to EMS personnel exposure reports per Solano County EMS Policy Memo #6170.

III. CONTINUING EDUCATION AND TAPE AUDIT REVIEW

- A. Base Hospitals servicing Solano County shall provide tape audit reviews and continuing education as specified in Memorandum of Agreement (MOA)/Contract.
- B. Base Hospitals servicing Solano County shall develop and properly orient all hospital employees who carry on EMS business over MED-NET or MED-COMM/Communications to include:
 - 1. Radio Operations
 - 2. Run documentation
 - 3. Base Hospital Communications
 - 4. Initiation of a retrospective audit of any EMS call in which care is in question.

IV. CERTIFICATION

All MICN's employed by the Base Hospitals servicing Solano County shall maintain Advanced Cardiac Life Support (ACLS) Certification. The Base Hospital shall retain on file at all times, copies of the current and valid certifications of all MICN's performing services under this agreement..

V. PREPARATION FOR MULTI-CASUALTY AND HAZ-MAT RESPONSE

Base Hospitals servicing Solano County shall train all MICN's in their respective roles and responsibilities under the County Multi-Casualty Incident Plan (MCI Plan) which is on file at the County EMS Agency.

VI. QUALITY IMPROVEMENT ACTIVITY REPORTS

Base Hospitals servicing Solano County shall submit the following elements of their Q.I. activities on a monthly basis (unless otherwise indicated) to the EMS Agency:

- A. Audit elements as described in Section II, paragraph 1, items a – 3.
- B. Unusual Occurrence Forms (within 5 days of being notified by the EMS Agency).
- C. Attendance Roster of EMS tape reviews.
- D. MCI/Disaster Incident Command System (ICS) Patient Logs for follow-up of mass casualty situations (within 72 hours of the event).
- E. Participate in the annual hospital facility assessment survey as required by 1797.103 of the Health & Safety Code.



Health Services Department

Public Health Division

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2202

EFFECTIVE DATE: 07/01/92

APPROVED BY:

Thomas L. Charron
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REVIEWED BY:

Steven J. Kaminski
STEVEN J. KAMINSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

AUTHORITY:

HEALTH AND SAFETY CODE 1797.204, 1797.220;
ADMINISTRATIVE CODE, TITLE 22, DIVISION 9,
CHAPTER 4, SECTION 100141.2, 100166.

SUBJECT: QUALITY ASSURANCE PLAN - RECEIVING HOSPITALS

PURPOSE/POLICY:

To establish specific Q.A. requirements for the Receiving Hospital components of the Solano EMS System.

I. WRITTEN AGREEMENT

- A. By December 31, 1992 each Receiving Hospital shall have a written agreement to participate in the County's ALS program and to comply with all applicable State Regulations and local Policies and Procedures including participation in the Local EMS Agency's Q.A. System.
- B. By July 1, 1992 each Receiving Hospital shall implement a comprehensive quality assurance program designed to interface with the Local EMS Agency's Quality Assurance Program and approved by the Local EMS Agency.

II. MEDICAL QUALITY ASSURANCE RESPONSIBILITIES

Receiving Hospitals shall have one physician, registered nurse, or EMT-P employed to implement and supervise their on-going internal Quality Assurance Program as related to EMS. This individual shall be approved by the Local EMS Agency to perform, at a minimum, the following functions:

- A. Investigation of all unusual occurrences as identified by the EMS Agency. Investigations shall take no longer than 5 days and/or a mutually agreed time interval, the results shall be reported directly back to the EMS Agency.
- B. Assure attendance of Q. A. Personnel at PCC Meetings as required by EMS Agency.
- C. Other Q. A. requirements established by the Solano County Health Officer.
- D. Completion and timely submission (as determined by Base Hospital) of Receiving Hospital log information to the base hospitals referring patients to the facility.
- E. Timely submission and proper filing of the PCR completed by EMS personnel into the Receiving Hospital's Patient Medical Record.
- F. Infection Control – Receiving Hospitals servicing Solano County shall develop and strictly enforce policies for infection control and contaminated materials disposal to decrease the chance of communicable disease exposure as well as ensure appropriate follow-up to EMS personnel exposure reports per Solano County EMS Policy Memo #6170.

III. TRAINING AND EDUCATION

Training & Education – Receiving Hospital under contract to Solano County EMS shall train all EMS personnel in their respective roles and responsibilities under the County Multi-Casualty Incident Plan (MCI Plan) which is on file at the County EMS Agency, and prepare staff to function within the role defined in the MCI.

IV. PRODUCTIVITY REPORTS

Receiving hospitals servicing Solano County shall submit the following Q. A. products and necessary statistical data on a monthly basis (unless otherwise indicated) to the EMS Office.

- A. Physician EMS Death Audits.
- B. Unusual Occurrences (within 7 days of being notified by the EMS Agency).
- C. Any Level, III Unusual Occurrence or Q. A. concern (immediately, or no later than 72 hours).
- D. Annually participate in the Hospital Facility Assessment Survey.



Health & Social Services Department

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2203

EFFECTIVE DATE: 23 OCTOBER, 1998

APPROVED BY:

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THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

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ALLEN MORINI, D.O., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

SUBJECT: QUALITY IMPROVEMENT PLAN - EMT-P SERVICE PROVIDER

AUTHORITY: HEALTH AND SAFETY CODE 1797.204, 1797.220;
ADMINISTRATIVE CODE, TITLE 22, DIVISION 9, CHAPTER 4,
SECTION 100141.2, 100166.

PURPOSE/POLICY

To establish specific Q.I. Requirements for the EMT-P Service Provider components of the Solano EMS System (ALS Ambulance Services and/or ALS non-transporting First Responders).

I. INTRODUCTION

- A. Each EMT-P Service Provider shall have a written agreement to participate in the County's ALS program and to comply with all applicable State Regulations and local Policies and Procedures including participation in the Local EMS Agency's Q.I. System.
- B. EMT-P Service Providers shall have and maintain a comprehensive Quality Improvement Program, that is approved by the LEMSA, designed to interface with the Local EMS Agency's Quality Improvement Program.

II. GOALS

- A. Delivery of optimal pre hospital care.
- B. Assure optimum clinical and field performance of caregivers within the provider's EMS System.

III. OBJECTIVES

- A. High quality care.
- B. Consistency in treatment by individuals within the provider organization.
- C. Open honest dialogue among EMS providers, base hospitals, and SEMSC.
- D. Accurate data collection.
- E. Individual and system improvement plan including tracking and trending.
- F. Provide and promote high quality continuing education.
- G. 100% compliance with licensure, accreditation, and certification requirements.
- H. Promote customer focus.
- I. EMS providers will have active involvement in the committee process.

IV. EMT-P SERVICE PROVIDER QUALITY IMPROVEMENT ACTIVITIES

The EMT-P Service Provider shall have one currently California licensed physician, registered nurse, or EMT-P employed to implement and supervise their on-going Quality Improvement Plan. Individuals currently serving in the role of the Q.I Coordinator as of October 15, 1998 will be permitted to continue in their role. This individual (Q.I. Coordinator) shall be approved by the Local EMS Agency and will insure the following functions at a minimum occur:

- A. Audits of pre hospital calls that will include, but will not be limited to, the following:
 - 1. Random sampling of calls to audit the elements established by the CQI Committee.

2. All calls where patients were examined by responding EMT/EMT-P's but resulted in non-transport to a medical facility.
 3. Patient contact by Air Ambulance / Rescue provider.
 4. All Mass Casualty Incidents and Hazardous Materials Incidents.
 5. Deviation from protocols for patients who present pulseless and apneic, specifically pertaining to SEMSC Policy #6150 and #6160.
 6. Unexpected deterioration of a patient while enroute to a receiving hospital.
 7. All calls requiring use of Specialized Procedures also to include patients that are intubated.
 8. All calls where scene and transport times are beyond established criteria.
 9. All unsuccessful intubations.
 10. All calls involving ALS intervention where Base Contact was not made.
 11. Specific review of a call as requested by the local EMS Agency, Base Hospital or the provider.
- B. Summary reports are to be submitted prior to the thirtieth (30th) day of the following month to the EMS Agency and appropriate Base Hospital. ALS providers can use an approved EMT-P Service Provider Audit Form (Attachment 1) or an approved equivalent to review Patient Care Reports. The Local EMS Agency will, with reasonable notice, perform periodic on-site evaluations of audit forms and pre hospital care records.
- C. Investigations of unusual occurrences as referred by the EMS Agency shall take no longer than ten (10) days and/or a mutually agreed time interval. The results shall be reported directly back to the EMS Agency.
- D. Monthly review of pre-hospital medical records to insure that PCR's are completed correctly and medical records are appropriately distributed. At a minimum, the review shall include EMS-MIS productivity reports documenting the minimum data elements required to be present on the PCR.
- E. Monitoring the success rate of each EMT-P of the following medical procedures:
1. Intubation.

2. Intravenous lines.

- F. Perform orientations for new EMT-1's and EMT-P's employed by the Service Provider, and submit an orientation form (Policy #3600) signed by orientor and orientee.
- G. Assure attendance of appropriate personnel at CQI and PCC meetings as required by the EMS Agency.
- H. Review unit activation, dispatch and response time intervals, investigate all performance failures, and submit findings to the EMS Office weekly for each case that is identified as beyond the threshold of performance expectation.
- I. Other Q.I. requirements established by the EMS Agency Medical Director.
- J. The EMT-P Service Provider shall notify the EMS Agency in writing of changes in the Q.I. coordinator position with two (2) weeks advance notice.

V. TRAINING/EDUCATION/CERTIFICATION/ACCREDITATION

- A. Record Keeping - The EMT-P Service Provider shall maintain records of current required certificates / licenses and mandatory training programs for its EMT-1 and EMT-P employees.
- B. Field Care Audits - The EMT-P Service Provider shall cooperate with the Base Hospitals and the EMS Agency in their role with field care audits.
- C. Mandatory Education for Local EMT-P Accreditation - EMT-P Service Provider will notify EMT-Ps of mandatory education programs.
- D. Personnel Investigation by EMS Agency - EMT-P Service Provider will cooperate fully with the Solano EMS Agency in any investigation involving the Service Provider in accordance with Policy #2310 and the California Health and Safety Code.
- E. Certification - All EMT-Ps employed by EMT-P Service Provider shall maintain Basic and Advanced Cardiac Life Support (ACLS) recognition certification based upon guidelines set forth by the JAMA

VI. MANAGEMENT INFORMATION SYSTEM (MIS)

EMT-P Service Provider shall have and maintain an EMS Management

Information System database compatible with and approved by the Solano EMS Agency. At a minimum the Q.I. Coordinator will be the assigned liaison with the EMS Agency for MIS.

- A. The data system must have the capability to generate an array of routine and special report as specified by the Solano EMS Agency.
- B. Provision must be made to permit secure electronic transfer of data, from Provider's computer system to the IBM compatible computers at the Local Emergency Medical Services Agency
- C. The minimal statistical data to be maintained by the EMT-P Service Provider's Management Information System (MIS) are the mandatory data elements of the Solano PCR and other data elements that may be required by the EMS Medical Director.
- D. Productivity Reports. ALS Service Providers servicing Solano County shall submit the following Q.I. products and necessary statistical data on a monthly basis (unless otherwise indicated) to the EMS Office or appropriate hospital.
 - 1. EMS copies of PCR's and electronic transfer of the previous PCR's entered in the provider's MIS. Submission of the previous month shall be submitted no later than the thirtieth (30) day of the following month.
 - 2. Receiving hospital copies of the ambulance PCR (**immediately, no later than 24 hours**)
 - 3. Base hospital copies of the ambulance PCR (**immediately, no later than seven (7) days**).
 - 4. Response time performance failure reports within three (3) days of the incident.
 - 5. Unusual occurrence submitted by the EMS Agency **within 7 days of receipt of the said notification, unless by mutual agreement between Q.I. staff and EMS staff that a different time interval is needed.**



Health Services Department

Public Health Division

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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2204

EFFECTIVE DATE: 07/01/92

APPROVED BY:

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REVIEWED BY:

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AUTHORITY: HEALTH AND SAFETY CODE 1797.204, 1797.220;
ADMINISTRATIVE CODE, TITLE 22, DIVISION 9,
CHAPTER 4, SECTION 100141.2, 100166.

SUBJECT: QUALITY ASSURANCE PLAN - BLS AMBULANCE SERVICE
PROVIDERS

PURPOSE/POLICY:

To establish specific Q.A. Requirements for the BLS Ambulance Service Provider components of the EMS System.

I. WRITTEN AGREEMENT

- A. On or before December 31, 1992, each BLS Ambulance Service Provider shall have a written agreement to participate in the County's EMS System program and to comply with all applicable State Regulations and local Policies and Procedures including participation in the Local EMS Agency's Q.A. System.
- B. By July 1, 1993, each BLS Ambulance Service Provider shall have a quality assurance program approved by the Local EMS Agency.

II. MAINTENANCE REQUIREMENTS

BLS Ambulance Service Providers shall maintain a comprehensive Quality Assurance Program designed to interface with the Local EMS Agency's Quality Assurance Program.

III. MEDICAL QUALITY ASSURANCE PERSONNEL

The BLS Ambulance Service Provider shall have one physician, registered nurse, EMT-P, EMT-I or other appropriately credentialed professional employed to implement and supervise their on-going internal Quality Assurance Program. This individual shall be approved by the Local EMS Agency to perform, at a minimum, the following functions:

- A. Audit of all calls where patients were examined by responding EMTs but resulted in non-transport to a medical facility. These written audits are to be submitted monthly, back to the Local EMS Agency.
- B. Investigation of all unusual occurrences as identified by the EMS Agency. Investigations shall take no longer than 5 days and/or a mutually agreed time interval. The results shall be reported directly back to the EMS Agency.
- C. Weekly review of prehospital medical records to insure PCR's are completed correctly and medical records are appropriately distributed.
- D. Perform all orientations for all new EMTs employed by the Service Provider, and submit an orientation form (Policy Memo #3600) signed by orientor and orientee. Until such time that the Prehospital Coordinator of the EMS Agency has reviewed and approved the recommendation, the Service Provider Q. A. Personnel shall insure that the new employee will not be assigned as the sole EMT for a BLS Ambulance Unit.
- E. Assure attendance of appropriate personnel at PCC meetings as required by the EMS Agency.
- F. Infection Control – BLS Ambulance Service Provider shall develop and strictly enforce policies for infection control and contaminated materials disposal to decrease the chance of communicable disease exposure.
- G. Q. A. Personnel shall weekly review ambulance activation, dispatch and response time intervals, investigate all ambulance performance failures, and submit findings to the EMS Office weekly for each case that is identified as beyond the threshold of performance expectation.
- H. Assure all EMTs are in compliance with Solano County HAZ-MAT Training Requirements and any other special educational programs required by the Solano County Health Officer.
- I. Other Q. A. requirements established by the Solano County Health Officer.

IV. TRAINING/EDUCATION/CERTIFICATION/ACCREDITATION

- A. Field Training Officer/Preceptor – BLS Ambulance Service Provider shall designate a minimum of one (1) field training officer for 25 FTE employees who shall function as trainers and perform other duties on behalf of BLS Ambulance Service Provider.
- B. Continuing Education Records – The BLS Ambulance Service Provider shall maintain records of continuing education and mandatory training programs for its BLS Ambulance Service employees.
- C. Field Care Audits – The BLS Ambulance Service Provider shall work cooperatively with the Base Hospitals and the EMS Agency in identifying and assisting with field care audits and ensure successful completion of diversion monitoring, re-education and/or recommended corrective actions as indicated.

- D. Investigation by EMS Agency – BLS Ambulance Service Provider will cooperate fully with the Solano County EMS Agency in the coordination of any investigation of an employee of the Service Provider with the Solano County EMS Agency or any Solano Base Hospital.
- E. Driver Training/Map Reading – The Solano County EMS Agency requires the BLS Ambulance Service Provider to maintain a driver training program. The program, the number of instruction hours, and the program's system for integration into the Service Provider's operations (e.g., accident review boards, impact of accidents on employee performance reviews, and compensation, etc.) will be reviewed and subject to approval of the Solano County EMS Agency. Map reading must be an integral part of each driver and attendant's training.
- F. Certification – All EMTs employed by BLS Ambulance Service Provider shall maintain Basic Cardiac Life Support (BCLS) Certification. The Service Provider shall retain on file at all times, copies of the current and valid certifications of all EMT-Is performing services under this agreement.
- G. Company Orientation and ON-Going Preparedness – BLS Ambulance Service Provider shall properly orient all field personnel before assigning them to respond to emergency medical requests. Such orientation shall include, at a minimum, Service Provider Agency Policies and Procedures; EMS System overview; EMS Policies and Procedures; radio communications with and between the Service Provider Agency, Base Hospital, receiving hospitals, and county Communications Centers; map reading skills including key landmarks, routes to hospitals and other major receiving facilities within the County and in surrounding areas; and ambulance and equipment utilization and maintenance.
- H. Preparation for Multi-Casualty and HAZ-MAT Response – BLS Ambulance Service Provider shall train all ambulance personnel and supervisory staff in their respective roles and responsibilities under the County Multi-Casualty Incident Plan (MCI Plan) which is on file at the County EMS Agency, and prepare them to function as the medical portion of the Incident Command system.
- I. Assaultive Behavior Management Training – BLS Ambulance Service Provider shall provide ambulance personnel with the training, knowledge, understanding and skills to effectively manage patients with psychiatric, drug/alcohol or other behavioral or stress related problems, as well as difficult or potentially difficult scenes on an on-going basis. Emphasis shall be on techniques for establishing a climate conducive to effective field management, and for preventing the escalation of potentially volatile situations.
- J. Management Information Systems (MIS) – BLS Ambulance Service Provider shall have and maintain an EMS Management Information System data base as specified by Solano County.
 - 1. EMS Providers will be responsible for all data entry to include all pertinent information (e.g., time of; call receipt, ambulance dispatch, ambulance underway, arrival at scene, depart scene, arrival at hospital, time available for another call), dry runs, and all information from the County Prehospital Care Report (PCR) forms for patients transported and those patients not transported which is required by the EMS Office.

2. The data system must have the capability to generate an array of routine and special reports as specified by the Solano County EMS Agency.
3. Provision must be made to permit access to and electronically transfer data in the data base, from Provider's computer system to the IBM compatible computers at the Local Emergency Medical Services Agency. The computer software used for the data base and report generation and instruction in its use shall be made available to the County EMS Office without charge or cost.
4. BLS Ambulance Service Provider may wish Solano County EMS Office staff to enter data at the Local EMS Agency. The Local EMS Agency may elect to perform this service and bill the BLS Ambulance Service Provider at a mutually agreed upon rate approved by the Board of Supervisors.
5. The minimal statistical data to be maintained by the BLS Ambulance Service Provider's Management Information System (MIS) are the mandatory data elements of the Solano PCR and other data elements which may be required by the EMS Medical Director.

V. PRODUCTIVITY REPORTS

BLS Ambulance Service Providers servicing Solano County shall submit the following Q. A. products and necessary statistical data on a monthly basis (unless otherwise indicated) to the EMS Office or appropriate hospital.

- A. EMS copies of PCRs and electronic transfer of the previous PCRs entered in the provider's MIS (submission of the previous month shall be submitted no later than the tenth (10th) day of the following month).
- B. Receiving hospital copies of the ambulance PCR (immediately, no later than 24 hours).
- C. Any Level III counseling form and/or Q. A. concern (immediately, no later than 24 hours).
- D. Monthly audit results of AMA calls.
- E. Ambulance response time performance failure reports within three (3) days of the incident.
- F. Unusual Occurrences submitted by the EMS Agency within 7 days of receipt of the said notification unless by mutual agreement between Q.A. Staff and EMS staff that a different time interval is needed.
- G. Quarterly audits of BLS activities (submitted 15 days after each calendar quarter; i.e., March, June, September, December).
- H. EMT-D audit and medical records.



Health Services Department

Public Health Division

Donald R. Rowe, Director

Thomas L. Charron, M.D., M.P.H.
Health Officer
Assistant Director

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2205

EFFECTIVE DATE: 07/01/92

APPROVED BY:

Thomas L. Charron
THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Steven J. Chinski
STEVEN J. CHINSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

AUTHORITY: HEALTH AND SAFETY CODE 1797.204, 1797.220;
ADMINISTRATIVE CODE, TITLE 22, DIVISION 9,
CHAPTER 4, SECTION 100141.2, 100166.

SUBJECT: QUALITY ASSURANCE PLAN - PUBLIC SAFETY AND/OR EMT-1
FIRST RESPONDERS WITHIN SOLANO COUNTY

PURPOSE/POLICY:

To establish specific Q.A. Requirements for the Public Safety and/or EMT-1 First Responders within Solano County.

I. WRITTEN AGREEMENT

- A. On or before December 31, 1992 each First Responder Agency shall have a written agreement to participate in the County's ALS program and to comply with all applicable State Regulations and local Policies and Procedures including participation in the Local EMS Agency's Q.A. System.
- B. By July 1, 1993 each First Responder Agency shall have a quality assurance program approved by the Local EMS Agency.

II. MAINTENANCE REQUIREMENTS

First Responder Agencies shall maintain a comprehensive Quality Assurance Program designed to interface with the Local EMS Agency's Quality Assurance Program.

III. MEDICAL QUALITY ASSURANCE PERSONNEL

The Basic Life Support First Responder Service Providers shall have one physician, registered nurse, EMT-P, EMT-I, or other appropriately credentialed medical professional employed to implement and supervise their on-going internal Quality Assurance Program. This individual shall be approved by the Local EMS Agency to perform, at a minimum, the following functions:

- A. Retrospective audit of all BLS calls where patients are assessed according to Solano County EMS BLS Audit Criteria.
- B. Investigation of all unusual occurrences as identified by the EMS Agency. Investigations shall take no longer than 5 days and/or a mutually agreed time interval; the results shall be reported directly back to the EMS Agency.
- C. Assure attendance of appropriate personnel at PCC meetings as required by the EMS Agency.
- D. Infection Control – First Responder Agencies shall develop and strictly enforce policies for infection control and contaminated materials disposal to decrease the chance of communicable disease exposure.
- E. Audit submission – EMT-D First Responder Agencies shall submit all audit review information governing EMT-D service as outlined in Policy #3300.
- F. BLS Audit – First Responder Service Providers shall perform a medical retrospective BLS audit of EMS cases using audit standards prescribed by the EMS Medical Director and the Solano County Fire Chief's Association.
- G. MCI Evaluation – First Responder Agencies who institute Incident Command at the scene of an MCI shall submit to Solano County EMS an MCI Evaluation Form.
- H. Assure EMT compliance with Solano County Haz-Mat Training Requirements and any other special educational programs required by the Solano County Health Officer.
- I. Other Q. A. requirements established by the Solano County Health Officer.

IV. TRAINING/EDUCATION/CERTIFICATION

- A. Field Training Officer – First Responder Service Provider shall designate a minimum of one (1) Field Training Officer for 25 FTE employees who shall function as trainers and perform other duties on behalf of First Responder Service Provider.
- B. Continuing Education Records – The EMT Service Provider shall maintain records of continuing education and mandatory training programs for its EMT employees, as mandated by California Code, Title 22 and/or Public Safety First Responders.
- C. Field Care Audits – The EMT Service Provider shall work cooperatively with the Base Hospitals and the EMS Agency in identifying and assisting with field care audits.

- D. EMT Investigation by EMS Agency – First Responder Service Provider will cooperate fully with the Solano County EMS Agency in the coordination of any investigation of an employee of the Service Provider with the Solano County EMS Agency or any Solano Base Hospital.
- E. Certification – All EMTs and/or Public Safety First Responders employed by EMT Service Provider shall maintain Basic Life Support (BLS) Certification. The Service Provider shall retain on file at all times, copies of the current and valid certifications of all Responders performing services.
- F. Company Orientation and On-Going Preparedness – First Responder Service Provider shall properly orient all field personnel before assigning them to respond to emergency medical requests. Such orientation shall include, at a minimum, Service Provider Agency Policies and Procedures; EMS System overview; EMS Policies and Procedures; receiving hospitals, and County Communications Centers; map reading skills including key landmarks, routes to difficult access facilities (e.g., jails, military bases, etc.) within the County and in surrounding areas.
- G. Preparation for Multi-Casualty and Haz-Mat Response – First Responder Service Provider shall train all personnel and supervisory staff in their respective roles and responsibilities under the County Multi-Casualty Incident Plan (MCI Plan) which is on file at the County EMS Agency, and prepare them to function as the medical portion of the Incident Command System, if needed.
- H. Assaultive Behavior Management Training – First Responder Service Provider shall provide personnel with the training, knowledge, understanding and skills to effectively manage patients with psychiatric, drug/alcohol or other behavioral or stress related problems, as well as difficult or potentially difficult scenes on an on-going basis. Emphasis shall be on techniques for establishing a climate conducive to effective field management, and for preventing the escalation of potentially volatile situations.

V. PRODUCTIVITY REPORTS

First Responder Service Providers servicing Solano County shall submit the following Q. A. products and necessary statistical data on a monthly basis (unless otherwise indicated) to the EMS Office or appropriate hospital.

- A. Quarterly audits of BLS activities (submitted quarterly, 15 days after the end of each calendar quarter; i.e., March, June, September, December).
- B. EMT-D Audit and medical record (electronic transfer of MIS and audit summary data is an acceptable equivalent).
- C. Receiving hospital copy of the PCR for any EMT-D case (immediately, or within 72 hours).
- D. MCI/Medical Disaster Summary Report (within 72 hours of any declared MCI in which First Responder service participated).
- E. Any Level III Q. A. concern (immediately, or within 72 hours of incident).

- F. Unusual occurrence submitted to service provider by the EMS Agency (within 7 days of receipt of said notification).
- G. Bi-annual plan for EMT continuing educational programs for approval with appropriate EMT-1 attendance records and documentation of acceptable skill performance of EMTs electing to be recertified by the EMS Office



Health Services Department

Public Health Division

Donald R. Rowe, Director

Thomas L. Charron, M.D., M.P.H.
Health Officer
Assistant Director

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2206

EFFECTIVE DATE: 07/01/92

APPROVED BY:

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

STEVEN J. KUMPSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

AUTHORITY:

HEALTH AND SAFETY CODE 1797.204, 1797.220;
ADMINISTRATIVE CODE, TITLE 22, DIVISION 9,
CHAPTER 4, SECTION 100141.2, 100166.

SUBJECT: QUALITY ASSURANCE PLAN - EMS AIRCRAFT

PURPOSE/POLICY:

To establish specific Q.A. Requirements for the EMS Aircraft components of the Solano EMS System.

I. WRITTEN AGREEMENT

- A. By December 31, 1992, each EMS Aircraft Service Provider shall have a written agreement to participate in the County's ALS program and to comply with all applicable State Regulations and local Policies and Procedures including participation in the Local EMS Agency's Q.A. System.
- B. By July 1, 1993, each EMS Aircraft Service Provider shall have a quality assurance program approved by the Local EMS Agency.

II. MAINTENANCE REQUIREMENT

EMS Aircraft Service Providers shall maintain a comprehensive Quality Assurance Program designed to interface with the Local EMS Agency's Quality Assurance Program.

III. MEDICAL QUALITY ASSURANCE PERSONNEL

The EMS Aircraft Service Providers shall have one physician, registered nurse, EMT-P, or other appropriately credentialed medical professional employed to implement and supervise their on-going internal Quality Assurance Program. This individual shall be approved by the Local EMS Agency to perform, at a minimum, the following functions:

- A. Audit of all calls where patients were examined by responding EMS Aircraft but resulted in non-transport to a medical facility. These written audits are to be submitted monthly back to the Local EMS Agency.
- B. Investigation of all unusual occurrences as identified by the EMS Agency. Investigations shall take no longer than 5 days and/or a mutually agreed time interval; the results shall be reported directly back to the EMS Agency.
- C. Assure attendance of appropriate personnel at PCC meetings as required by the EMS Agency.
- D. Infection Control – EMS Aircraft Service Provider shall develop and strictly enforce policies for infection control and contaminated materials disposal to decrease the chance of communicable disease exposure.
- E. Q. A. personnel shall monthly review helicopter activation, dispatch and response time intervals, submit findings to the EMS Office monthly for each case that is identified as beyond the threshold of performance expectation.
- F. Other Q. A. requirements established by the Solano County Health Officer.

IV. TRAINING/EDUCATION/CERTIFICATION/ACCREDITATION

- A. EMS Aircraft Personnel – EMS Aircraft Service Provider will cooperate fully with the Solano County EMS Agency in the coordination of any investigation of an employee of the Service Provider with the Solano County EMS Agency or any Solano Base Hospital
- B. Certification – All personnel employed by EMS Aircraft Service Provider shall maintain Advanced Cardiac Life support (ACLS) and Basic Life Support (BLS) certification. The Service Provider shall retain on file at all times, copies of the current and valid certifications of all personnel performing services under this agreement.
- C. Preparation for Multi-Casualty and Haz-Mat Response –EMS Aircraft Service Provider shall train all personnel and supervisory staff in their respective roles and responsibilities under the County Multi-Casualty Incident Plan (MCI Plan) which is on file at the County EMS Agency, and prepare them to function as the medical portion of the Incident Command System, if needed.

V. PRODUCTIVITY REPORTS

ALS Service Providers servicing Solano County shall submit the following Q. A. products and necessary statistical data on a monthly basis (unless otherwise indicated) to the EMS Office or appropriate hospital.

- A. EMS copies of PCRs and electronic transfer of the previous PCRs entered in the provider's MIS (submission of the previous month shall be submitted no later than the tenth (10th) day of the following month).
- B. Receiving hospital copy of the PCR for any EMT-D case (immediately, or within 72 hours).
- C. Any Level III counseling form and/or Q.A. concern (immediately, no later than 24 hours).
- D. Audit results of AMA calls.
- E. Unusual occurrence submitted to service provider by the EMS Agency (within 7 days of receipt of said notification).



Health Services Department

Public Health Division

Donald R. Rowe, Director

Thomas L. Charron, M.D., M.P.H.
Health Officer
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EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2207

EFFECTIVE DATE: 07/01/92

APPROVED BY:

Handwritten signature of Thomas L. Charron.

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

Handwritten signature of Steven J. Kaniwsky.

STEVEN J. KANIWSKY, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

AUTHORITY:

HEALTH AND SAFETY CODE 1797.204, 1797.220;
ADMINISTRATIVE CODE, TITLE 22, DIVISION 9,
CHAPTER 4, SECTION 100141.2, 100166.

SUBJECT: QUALITY ASSURANCE PLAN - EMS ADVISORY COMMITTEES

PURPOSE/POLICY:

To establish specific Q.A. Requirements for EMS Committee components of the Solano EMS System.

I. EMERGENCY MEDICAL CARE COMMITTEE (EMCC)

- A. By April each year the EMCC shall submit to the Solano County Board of Supervisors the EMS Agency Annual Report.
- B. Shall on a quarterly basis review productivity statistics as submitted by Solano County EMS Agency Personnel.

II. PREHOSPITAL CARE COMMITTEE (PCC)

- A. Shall retrospectively review all MCIs utilizing the MCI evaluation form and any other documentation available (i.e. PCRs, Triage Tags, Unusual Occurrence Forms).
- B. Annually, at its July meeting, to review EMS aircraft utilization with EMS aircraft service providers authorized to function in Solano County.

III. PHYSICIAN FORUM *

- A. Shall retrospectively review all cases as defined by Solano County EMS Death Audits.
- B. Meet at least bi-monthly to discuss Q. A. issues surrounding medical control.

* All activities conducted during Physician Forum meetings are undiscoverable by Solano County Board of Supervisors resolution #91-92.